

DEMO REFERRAL HOSPITAL BOARD

Procedures for the Prevention of Bribery and Corruption

Developed pursuant to section 9 of the Anti-Bribery Act (Cap. 79B), customised from the EACC Model Procedures (July 2024)

Institution	Demo Referral Hospital Board (DRHB), Institution ID DEMO-005
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Version / status	Version 1.0 - ACTIVE (no predecessor)
Approval	Approved by the Board at its 112th Meeting held on 26 August 2026, Resolution No. DRHB/BD/RES/2026/044
Effective from	1 September 2026
Effective to	Until superseded; scheduled review by 31 August 2028
Custodian	Mr. Kevin Odhiambo, Head, Legal Services; Designated Senior Officer (Anti-Bribery)

Evidence category: **Procedure evidence** (Sub-indicator 1). Submitted to the oversight portal on 29 September 2026, before the 30 September 2026 foundation deadline.

1.0 Institutional identity and scope

The Demo Referral Hospital Board is a (fictional) state corporation that runs a 450-bed national referral hospital providing specialist inpatient, outpatient, surgical, diagnostic and pharmacy services, with an establishment of 1,120 staff.

These Procedures apply to the Board, management, all employees (permanent, contract, locum and interns), agents, suppliers, contractors and any other third party that transacts with the Hospital. They cover both the core mandate (patient admission and bed allocation, theatre and specialist clinic booking, pharmacy dispensing and medical stores, patient billing and fee waivers) and all support services (procurement, human resource, finance, ICT, and medical equipment maintenance).

2.0 Commitment and objective

The Hospital maintains zero tolerance to bribery and corruption and is committed to the prevention, detection, deterrence, reporting of and appropriate action against bribery and other forms of corruption, in adherence to the Anti-Bribery Act, the Bribery Regulations 2022, the Attorney-General's Guidelines (2022) and these Procedures.

3.0 Procedure content

3.1 Relevant policies

These Procedures complement the Hospital's Code of Conduct and Ethics, Whistle-blower Protection Policy, Conflict of Interest Policy, Gifts and Hospitality Policy, Patient Fee Waiver Policy, Anti-Fraud Policy and the Public Procurement and Asset Disposal Act.

3.2 Bribery and corruption risk assessment

The Hospital shall identify, assess and map bribery and corruption risks across core and support functions, design controls, and monitor and review them. The assessment is overseen by top management through the Corruption Prevention Committee (CPC), conducted by the Integrity Assurance Officers (IAOs), reviewed at least annually and documented in a Risk Assessment Report and an Anti-Bribery and Corruption Mitigation Plan.

3.3 High-risk areas

Procurement of pharmaceuticals and medical consumables; approval of requisitions, evaluations and purchase orders; pharmacy dispensing and medical stores; patient admission and bed allocation; theatre and specialist clinic waiting lists; patient billing and fee waivers; recruitment and locum engagement; supplier payments; gifts and hospitality from suppliers; and conflicts of interest.

3.4 Managing third parties

All contracts above KES 500,000 shall include anti-bribery clauses and a conflict-of-interest declaration; prequalified suppliers complete a due-diligence questionnaire; these Procedures are shared with all third parties on engagement.

3.5 Specific controls for quarterly orders under framework agreements

Quarterly orders for pharmaceuticals and medical consumables under framework agreements shall be distributed among the prequalified suppliers in accordance with a supplier rotation list recommended by the Tender Committee and approved by the Accounting Officer. Each order lot is placed with the supplier next due on the list. Any deviation requires prior written approval of the Accounting Officer with reasons, is recorded in the rotation log and is reported to the CPC at its next meeting. From Q2, Internal Audit with the IAO Committee shall complete a quarterly exception review of the rotation control, and shall each quarter spot-check a sample of at least ten procurement approvals against the delegation of authority schedule, recording its findings for the CPC.

3.6 Reporting channels and processing of reports

Reports may be made in person to the Designated Senior Officer, in writing, to integrity@demo-005.example.invalid, through the reporting boxes at each clinic and ward block, through the anonymous web form, or directly to the Ethics and Anti-Corruption Commission through its official channels. Reports are acknowledged, recorded in the Reports Register, forwarded by the Designated Senior Officer to the enforcement structure and to the Commission within 24 hours, inquired into, and the outcome communicated to the Commission within 7 days of determination. The Annex I reporting form of the EACC Model Procedures is adopted without change.

3.7 Protection of whistle-blowers, informants and witnesses

The identity of reporters is kept confidential; retaliation is a disciplinary offence; anonymous reporting is permitted; reports of victimisation are handled by the Designated Senior Officer directly with the Accounting Officer.

3.8 Communication and training

These Procedures are published on the staff intranet and website. All staff in high-risk functions shall be trained on them as named in the Mitigation Plan; all other staff receive annual sensitisation.

3.9 Monitoring, evaluation and review

The CPC monitors implementation quarterly. These Procedures are reviewed every two years or earlier on a material change in risk, law or EACC guidance, using the Annex 2 compliance checklist of the EACC Model Procedures.

4.0 Designated implementation responsibility

Structure	Holder	Responsibility
Top-level management	Dr. Wanjiru Kimani, Chief Executive Officer and Accounting Officer	Overall accountability; chairs the CPC; approves the risk assessment, mitigation plan and supplier rotation list; provides resources.
Designated Senior Officer	Mr. Kevin Odhiambo, Head, Legal Services (appointed by letter DRHB/CEO/ADM/2026/207 of 1 September 2026)	In charge of prevention of bribery; receives and forwards reports; protects whistle-blowers; sets up the enforcement structure.
Implementation structure	Corruption Prevention Committee (CPC)	Oversees execution of these Procedures; meets at least once each quarter; approves quarterly reports to the Commission.
Technical support	IAO Committee (Chair: Ms. Naomi Akinyi)	Conducts risk assessment; prepares the mitigation plan and quarterly progress reports; meets before each CPC meeting.
Enforcement structure	Staff Disciplinary Committee	Inquires into reports and recommends administrative action under the HR Policy.

5.0 Approval details

Approving authority	Board of Directors, Demo Referral Hospital Board
Meeting and resolution	112th Board Meeting, 26 August 2026; Resolution DRHB/BD/RES/2026/044 adopting these Procedures
Effective date	1 September 2026
Next review	By 31 August 2028

Name	Designation / role	Signature	Date
Prof. Hassan Abdi	Chairperson, Board of Directors		26 Aug 2026
Dr. Wanjiru Kimani	Chief Executive Officer and Accounting Officer; Secretary to the Board		26 Aug 2026

Declaration of commitment to implement the requirements of the Anti-Bribery Act, Regulations, Guidelines and these Procedures

The Demo Referral Hospital Board is committed to implement these Procedures on prevention of bribery and other forms of corruption developed pursuant to section 9 of the Anti-Bribery Act, Cap. 79B.

Name	Designation / role	Signature	Date
Dr. Wanjiru Kimani	Head of Entity: Chief Executive Officer		1 Sep 2026

Date of next review: 31 August 2028.